



Lata Mangeshkar Medical Foundation's

Deenanath Mangeshkar Hospital & Research Center

Erandawane, Pune-411004. Contact : 020 49153000 Ext.: 3175/3191/1039
Email : lab@dmhospital.org|Patient Portal : phr.dmhospital.org



DEPARTMENT OF PATHOLOGY

TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:OP0005
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260326BI0496
Category	:GNL	Ward/Room	:
		Referred By	:KURLEKAR UTKRANT ANANT

BIOCHEMISTRY (ROUTINE)

Blood Glucose Random		Sample Id		: 0659BI260326	
Collection DateTime	: 26/03/2026 12.54.51	Receiving	: 26/03/2026 12.55.18	Reporting DateTime	: 26/03/2026 16.37.31
Primary Sample Type	: Whole Blood/Urine	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
Blood Glucose Random	98	Hexokinase	mg %	70.0 - 140.0	Note: In an individual with classic symptoms of hyperglycemia or hyperglycemic crisis, a random plasma glucose; $\geq$ 200 mg/dL is S/O diagnosis of diabetes (ADA Guidelines 2024)

HIL INDEX-WNL
Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.
Interpretation: Increased in Diabetes Mellitus, stress (shock, burns), Acute pancreatitis, Cushing's syndrome, adrenalin injection, etc.Decreased in Islet cell tumour, glucagon deficiency, carcinoma of adrenal gland, Addison's disease, ketotic hypoglycaemia, etc.

***** End Of Report *****

Namita
DR. NAMITA MAHALLE
M.Sc., Ph.D.(Biochemistry),
Consultant Biochemist

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Note : Interpretation of Lab Results may vary in the light of clinical data.
Kindly correlate clinically & communicate your queries if any.





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MRD No	:332192	Visit Code	:OP0005
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260326BI0496
Category	:GNL	Ward/Room	:
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY(ROUTINE)

Liver Function Tests[Panel]		Sample Id	: 0658BI260326
Collection DateTime	: 26/03/2026 12.54.51	Receiving	: 26/03/2026 12.55.18
Primary Sample Type	: Serum	Reporting DateTime	: 26/03/2026 16.01.49
		Sample Suitability	: Suitable
		Result Status	: Result Certified

Test	Result	Method	Unit	Reference Interval
*Liver Function Test:				
Total Bilirubin	0.61	DMSO	mg %	0.2 - 1.2 Cord blood : < 2.0
Direct Bilirubin	0.24	DMSO	mg %	Upto 0.5
Indirect Bilirubin	0.37	Calculated	mg %	Upto 0.8
SGPT	11	(Kinetic)	IU/L	10.0 - 40.0
SGOT	9	(Kinetic)	IU/L	11.0 - 34.0
Serum Alkaline Phosphatase	120	PNP-AMP	IU/L	50.0 - 116.0
Total Proteins	5.15	Biuret	gm %	6.0 - 8.0
Serum Albumin	2.81	BCG	gm %	3.5 - 5.2
Serum Globulin	2.34	Calculated	gm %	2.0 - 3.5
Albumin: Globulin Ratio	1.2	Calculated	-	1.0 - 2.0

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Increased AST and ALT levels indicate liver damage, disease or muscle damage. Increased ALP indicate liver damage or blocked bile duct, or certain bone diseases. Elevated levels of bilirubin (jaundice) indicates liver damage or disease or certain types of anemia. Lower-than-normal levels of albumin and total protein may indicate liver damage or disease.High bilirubin and normal direct bilirubin indicate hemolysis. Similar situation is in neonates. Increased bilirubin and increased ALP with near normal AST and ALT suggests obstructive jaundice.

***** End Of Report *****

Hafita

DR. NAMITA MAHALLE
M.Sc., Ph.D.(Biochemistry),
Consultant Biochemist

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DEPARTMENT OF PATHOLOGY
TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	: 14/05/2026 16.09.30
MRD No	:332192	Visit Code	:OP0005
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260326BI0496
Category	:GNL	Ward/Room	:
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY (ROUTINE)

RFT[Panel]		Sample Id		: 0658BI260326	
Collection DateTime	: 26/03/2026 12.54.51	Receiving	: 26/03/2026 12.55.18	Reporting DateTime	: 26/03/2026 15.39.20
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
Blood Urea Level	15	(UV Kinetic)	mg %	19.44 - 44.08	
Creatinine::	0.85	(Alkaline picrate)	mg %	0.6 - 1.3	
eGFR	102.69	Calculated	ml/min	Normal/Minimal kidney damage:>=90 ml/min/1.73m2 Impairment : ml/min/1.73m2 Mildly decreased : 60-89 Mild to moderately decreased: 45-59 Moderately to severely decreased : 30-44 Severely decreased:15-29	
Serum Calcium	8.02	(Arsenazo III)	mg %	8.4 - 10.2	
Serum Phosphorus	3.43	(UV phosphomolybdate)	mg %	2.3 - 4.7	
Serum Uric Acid	6.72	(Uricase)	mg %	3.7 - 7.7	
Serum Sodium	139	ISE(Indirect)	mEq/L	136.0 - 145.0	
Serum Potassium	4.7	ISE(Indirect)	mEq/L	3.5 - 5.1	
Serum Chlorides	108	ISE(Indirect)	mEq/L	98.0 - 107.0	

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Serum urea is increased in gastrointestinal bleeding, dehydration, kidney disease, obstruction in the urinary tract, and shock. Decreased with liver failure, malnutrition.Serum creatinine increased in acute and chronic renal failures, shock, severe dehydration and muscular waste due to severe injury. Decreased in protein-calorie malnutrition. High total calcium may be due to Hyperthyroidism. Low total calcium is due to hypoparathyroidism, extreme deficiency in dietary calcium. Increase in phosphate is due to high intake of phosphate or vitamin D, hypocalcemia, hypoparathyroidism. Decreased in chronic use of antacids, lack of vitamin D. Uric acid is increased in gout, renal failure, leukemia, Lesch-Nyhan syndrome. Decreased in Wilson’s disease, Fanconi syndrome. Sodium is increased in water loss, dehydration, vomiting, diarrhea, burns. Decreased in use of diuretics, renal tubular acidosis. Potassium is elevated with increased supply of potassium as in potassium infusion, redistribution of potassium and decreased with chronic starvation, prolonged vomiting, diarrhea. Chloride is increased in renal tubular acidosis, acute renal failure, diabetes insipidus and decreased in excessive sweating, prolonged vomiting, persistent gastric secretion and cystic fibrosis.

***** End Of Report *****

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DR. NAMITA MAHALLE
M.Sc., Ph.D.(Biochemistry),
Consultant Biochemist

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MC-2177



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MRD No	:332192	Visit Code	:OP0005
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260326CP0112
Category	:GNL	Ward/Room	:
		Referred By	:DR. KURLEKAR UTKRANT

CLINICAL PATHOLOGY

Urine Routine (Panel)	Sample Id	: 0136CP260326
Collection DateTime : 26/03/2026 12.54.51	Receiving	: 26/03/2026 12.55.18
Primary Sample Type : Urine	Reporting DateTime	: 26/03/2026 14.15.46
	Sample Suitability	: Suitable
	Result Status	: Result Certified

Test	Result	Method	Unit	Reference Interval
<u>.Urine Chemistry</u>	.	Dipstik /chemical examination		
Quantity Examined	05		ml	
Colour	Yellow	Dipstik /chemical examination		Pale Yellow
Appearance	Clear	Dipstik /chemical examination		Clear
pH	5.5	Bromthymol blue,Methyl red		4.6 - 8.0
Specific Gravity	1.014	Bromthymol blue indicator		1.005 - 1.035
Protein	Negative	Tetrabromphenol blue		Negative
Glucose	Negative	Glucose oxidase peroxidase O-Tolidine hydrochlorid		Negative
Ketones	Negative	Sodium nitroprusside		Negative
Bilirubin	Negative	Diazonium salt		Negative
Urobilinogen	+	Diazonium salt	mg/dl	0.1 - 1.8 mg/dl : Normal >2 mg/dl : +,++,+++
Blood	Negative	Isopropylbenzol-hydroperoxide,Tetra methylbenzidine		Negative
Leucocyte Esterase	Negative	Carboxylic acid ester,Diazonium salt		Negative
Nitrite	Negative	Sulfanilic acid,Tetrahydrobenz ol(h) quinolon-3-ol		Negative
Ascorbic acid	Negative	2,6-dichloro-phenol-indophenol		Negative
<u>.Microscopic Examination</u>	.	Cuvette based imaging technology		
Pus Cells	0.00		/hpf.	0 - 5.00
Pus cells clumps	0.00		/hpf.	0 - 0.00
Red blood cells	0.00		/hpf.	0 - 2.00
Dysmorphic RBC	0.00		/hpf.	0 - 0.00
Non squamous epithelial cells	0.00		/hpf.	0 - 1.00
Squamous epithelial cells	1.00		/hpf.	0 - 1.00
<u>.Crystals</u>	.			
Crystals-	0.00		/hpf.	0 - 0.00
Calcium oxalate crystals	0.00	4/33	/hpf.	0 - 0.00



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Category	:GNL	Ward/Room	:
		Referred By	:DR. KURLEKAR UTKRANT

Triple phosphate crystals	0.00	/hpf.	0 - 0.00
Uric acid crystals	0.00	/hpf.	0 - 0.00
Calcium phosphate crystals	0.00	/hpf.	0 - 0.00
Amorphous material	0.00	/hpf.	0-0.00
.Cast	.		
Hyaline casts	0.00	/hpf.	0 - 2.00
Pathological casts	0.00	/hpf.	0 - 0.00
Granular casts	0.00	/hpf.	0 - 0.00
RBC casts	0.00	/hpf.	0 - 0.00
WBC casts	0.00	/hpf.	0 - 0.00
.Microorganisms	.		
Bacteria-	0.00	/hpf.	0 - 0.00
Bacteria rods	0.00	/hpf.	0 - 0.00
Bacteria cocci	0.00	/hpf.	0 - 0.00
Yeast -	0.00	/hpf.	0 - 0.00
Parasites-schistosoma Haematobium	0.00	/hpf.	0 - 0.00
.Others	..		
Mucus--	0.00	/hpf.	0 - 0.00
Spermatozoa	0.00	/hpf.	0 - 0.00
Note	.		

Limitations:
Bilirubin :False low or negative results may be simulated by large amount of vit C or by longer exposure of sample to direct light.
Urobilinogen : False positive results may be produced by beetroot consumption or metabolites of drugs eg. Phenazopyridine, p-aminobenzoic acid.
Glucose : High concentrations of ascorbic acid in urine may lead to false negative results.
Repeat the test 10hrs to one day after stopping the intake of Vit C.
Lysed RBCs are not detected by sediment analysis.
Nitrite: False positive results may occur in long standing urine samples where nitrite has been formed by contamination of the sample and in urine containing dyes (Beetroot, derivatives of pyridium) Negative result in presence of bacteriuria may be due to antibiotic treatment, high diuresis, high content of ascorbic acid or insufficient incubation of urine in bladder.
Leucocytes: False positive reaction may be caused by contamination of vaginal secretion.
Leucocyte esterase may be positive in absence of leucocytes if they have lysed.

***** End Of Report *****

M. Khatri:

DR.MONIKA KHATRI

MBBS, MD(Pathology)

Senior Registrar

Reg.No.: MP-18449

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MRD No	:332192	Visit Code	:OP0005
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260326HM0378
Category	:GNL	Ward/Room	:
		Referred By	:DR. KURLEKAR UTKRANT

HAEMATOLOGY

Haemogram[Panel]				Sample Id	: 0393HM260326
Collection DateTime		: 26/03/2026 12.54.51	Receiving		: 26/03/2026 12.55.18
Primary Sample Type		: Whole Blood	Sample Suitability		: Suitable
		Result Status		: Result Certified	
Test	Result	Method	Unit	Reference Interval	
Haemoglobin	12.3	SLS	gm %	13.0 - 18.0	
RBC Count	5.07	Impedance	Mill/Cmm	4.0 - 5.5	
PCV	42.3	Cumulative pulse height detection	%	40.0 - 50.0	
MCV	83.4	Calculated	fL	75.0 - 95.0	
MCH	24.3	Calculated	pg	25.0 - 32.0	
MCHC	29.1	Calculated	g/dl	30.0 - 35.0	
RDW	23.8	Calculated	%	11.6 - 14.8	
Total Leucocyte Count ..	13220	Fluoresence FCM	/Cmm	4000.0 - 10000.0 ***** FCM= Flow Cytometry	
<u>.Differential Count.</u>		.			
Neutrophils	75.7	Fluoresence FCM	%	40.0 - 75.0	
Lymphocytes-	13.1	Fluoresence FCM	%	20.0 - 45.0	
Eosinophils	1.7	Fluoresence FCM	%	1.0 - 6.0	
Monocytes	9.2	Fluoresence FCM	%	1.0 - 10.0	
Basophils	0.3	Fluoresence FCM	%	0.1 - 1.0	
Absolute Neutrophil Count (ANC)	10010	Fluoresence FCM	/Cmm	2000.0 - 7500.0	
Absolute Lymphocytes Count	1730	Fluoresence FCM	/Cmm	1500.0 - 4000.0	
Absolute Eosinophil Count (AEC)	220	Fluoresence FCM	/Cmm	40.0 - 400.0	
Absolute Monocytes Count	1220	Fluoresence FCM	/Cmm	200.0 - 800.0	
Absolute Basophil Count	40	Fluoresence FCM	/Cmm	0.1 - 100.0	
Platelet Count	449000	Impedance	/Cmm	150000.0 - 450000.0	
MPV	7.9	Calculated	fL	8.0 - 11.5	
<u>.Peripheral Smear</u>		.			
RBCs..-	Normocytic normochromic, moderate anisocytosis, few microcytic hypochromic red cells				
WBCs (Hgm)	Neutrophilic leucocytosis				
Platelet Morphology	Adequate				

NOTE : All abnormal haemogram are reviewed and confirm microscopically



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MRD No	:332192	Visit Code	:OP0005
DoB/Sex	:20/06/1978 [47Y 9M] : M	Ward/Room	:
Category	:GNL	Referred By	:DR. KURLEKAR UTKRANT
Lab Reference No	:20260326HM0378		

***** End Of Report *****

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MRD No	:332192	Visit Code	:OP0005
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260326HM0378
Category	:GNL	Ward/Room	:
		Referred By	:DR. KURLEKAR UTKRANT

HAEMATOLOGY

Prothrombin Time[Panel]		Sample Id	: 0394HM260326		
Collection DateTime	: 26/03/2026 12.54.51	Receiving	: 26/03/2026 12.55.18	Reporting DateTime	: 26/03/2026 15.37.44
Primary Sample Type	: Whole Blood	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
Prothrombin Time (PT)	12.3	POCD	sec	10.1 - 12.5	Method : Photo Optical Clot Detection. (POCD)
Mean Normal PT	11.3	POCD	sec		
INR	1.10	Calculated			

***** End Of Report *****

Vijaya Gadage
DR. VIJAYA GADAGE
M.D. D.N.B.(Pathology)
Fellowship in Hematopathology
Consultant Hematopathologist
Reg. No: 2004/02/0866

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DEPARTMENT OF PATHOLOGY
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Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260331BI0698
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

TUMOUR MARKERS

CEA (TUMOUR MARKERS)		Sample Id		: 0842BI260331	
Collection DateTime	: 31/03/2026 20.21.50	Receiving	: 31/03/2026 21.27.15	Reporting DateTime	: 01/04/2026 13.39.33
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
CEA	55.85	CMIA	ng/ml	Reference Interval Smoker : Upto 5 ng/ml Non smoker : Upto 3 ng/ml	

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Increased levels of CEA are found in colorectal or colon cancer, medullary thyroid carcinoma, breast cancer, cancer of the gastrointestinal tract, liver cancer, lung cancer, ovarian cancer, pancreatic cancer, prostate cancer

***** End Of Report *****


DR. SHARWARI NARAWADE
M.D.(Biochemistry), Associate Consultant
Reg. No: 2012030602

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DEPARTMENT OF TRANSFUSION MEDICINE

TEST REPORT

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MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260402BB0007
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BLOOD CENTRE

BLOOD GROUP AND SCREEN				Sample Id	: 0014BB260402
Collection DateTime	: 02/04/2026 02.50.29	Receiving	: 02/04/2026 02.50.37	Reporting DateTime	: 02/04/2026 09.52.11
Primary Sample Type	: Whole Blood	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
ABO	AB	Column Agglutination Technique			
Rh	POS				
Antibody Screen(3 Cell)	NEG				

***** End Of Report *****

R. R. Patil
DR.RUCHA PATIL
MBBS
Blood Transfusion Officer
Reg.No.: 20251011242

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Note : 1] This blood group has been reported from first sample.
Kindly send second sample for blood group confirmation.
2] For newborns only, kindly confirm blood group after 6 months of age.



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MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260404BI0301
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY (SPECIAL)

Lipase				Sample Id	: 0310BI260404
Collection DateTime	: 04/04/2026 09.15.34	Receiving	: 04/04/2026 10.35.58	Reporting DateTime	: 06/04/2026 15.15.20
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
Lipase	6	Kinetic colorimetric	U/L	<= 60.0 U/L	

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Increased in, acute pancreatitis, sudden inflammation of the pancreas, chronic pancreatitis, long-term inflammation of the pancreas, pancreatic pseudocyst, fluid-filled sac around the pancreas, pancreatic cancer, cholecystitis, inflammation of the gallbladder, ectopic pregnancy, mumps, salivary gland blockage, intestinal blockage. Decreased in, severe injury to the pancreas, high triglycerides, diabetes

***** End Of Report *****


DR. SHARWARI NARAWADE
M.D.(Biochemistry), Associate Consultant
Reg. No: 2012030602

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Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260404BI0301
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY(ROUTINE)

Liver Function Tests[Panel]	Sample Id	: 0310BI260404
Collection DateTime : 04/04/2026 09.15.34	Receiving	: 04/04/2026 10.35.58
Primary Sample Type : Serum	Reporting DateTime	: 04/04/2026 13.56.30
	Sample Suitability	: Suitable
	Result Status	: Result Certified

Test	Result	Method	Unit	Reference Interval
*Liver Function Test:	.			
Total Bilirubin	0.47	DMSO	mg %	0.2 - 1.2 Cord blood : < 2.0
Direct Bilirubin	0.21	DMSO	mg %	Upto 0.5
Indirect Bilirubin	0.26	Calculated	mg %	Upto 0.8
SGPT	15	(Kinetic)	IU/L	10.0 - 40.0
SGOT	14	(Kinetic)	IU/L	11.0 - 34.0
Serum Alkaline Phosphatase	86	PNP-AMP	IU/L	50.0 - 116.0
Total Proteins	4.14	Biuret	gm %	6.0 - 8.0
Serum Albumin	2.62	BCG	gm %	3.5 - 5.2
Serum Globulin	1.52	Calculated	gm %	2.0 - 3.5
Albumin: Globulin Ratio	1.72	Calculated	-	1.0 - 2.0

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Increased AST and ALT levels indicate liver damage, disease or muscle damage. Increased ALP indicate liver damage or blocked bile duct, or certain bone diseases. Elevated levels of bilirubin (jaundice) indicates liver damage or disease or certain types of anemia. Lower-than-normal levels of albumin and total protein may indicate liver damage or disease.High bilirubin and normal direct bilirubin indicate hemolysis. Similar situation is in neonates. Increased bilirubin and increased ALP with near normal AST and ALT suggests obstructive jaundice.

***** End Of Report *****

DR. SHARWARI NARAWADE
M.D.(Biochemistry), Associate Consultant
Reg. No: 2012030602

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Email : lab@dmhospital.org|Patient Portal : phr.dmhospital.org



DEPARTMENT OF PATHOLOGY
TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260404BI0301
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY (ROUTINE)

RFT[Panel]		Sample Id		: 0310BI260404
Collection DateTime	: 04/04/2026 09.15.34	Receiving	: 04/04/2026 10.35.58	Reporting DateTime : 04/04/2026 13.49.39
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status : Result Certified
Test	Result	Method	Unit	Reference Interval
Blood Urea Level	39	(UV Kinetic)	mg %	19.44 - 44.08
Creatinine::	0.80	(Alkaline picrate)	mg %	0.6 - 1.3
eGFR	110.13	Calculated	ml/min	Normal/Minimal kidney damage:>=90 ml/min/1.73m2 Impairment : ml/min/1.73m2 Mildly decreased : 60-89 Mild to moderately decreased: 45-59 Moderately to severely decreased : 30-44 Severely decreased:15-29
Serum Calcium	7.48	(Arsenazo III)	mg %	8.4 - 10.2
Serum Phosphorus	2.32	(UV phosphomolybdate)	mg %	2.3 - 4.7
Serum Uric Acid	5.25	(Uricase)	mg %	3.7 - 7.7
Serum Sodium	135	ISE(Indirect)	mEq/L	136.0 - 145.0
Serum Potassium	4.2	ISE(Indirect)	mEq/L	3.5 - 5.1
Serum Chlorides	105	ISE(Indirect)	mEq/L	98.0 - 107.0

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Serum urea is increased in gastrointestinal bleeding, dehydration, kidney disease, obstruction in the urinary tract, and shock. Decreased with liver failure, malnutrition.Serum creatinine increased in acute and chronic renal failures, shock, severe dehydration and muscular waste due to severe injury. Decreased in protein-calorie malnutrition. High total calcium may be due to Hyperthyroidism. Low total calcium is due to hypoparathyroidism, extreme deficiency in dietary calcium. Increase in phosphate is due to high intake of phosphate or vitamin D, hypocalcemia, hypoparathyroidism. Decreased in chronic use of antacids, lack of vitamin D. Uric acid is increased in gout, renal failure, leukemia, Lesch-Nyhan syndrome. Decreased in Wilson’s disease, Fanconi syndrome. Sodium is increased in water loss, dehydration, vomiting, diarrhea, burns. Decreased in use of diuretics, renal tubular acidosis. Potassium is elevated with increased supply of potassium as in potassium infusion, redistribution of potassium and decreased with chronic starvation, prolonged vomiting, diarrhea. Chloride is increased in renal tubular acidosis, acute renal failure, diabetes insipidus and decreased in excessive sweating, prolonged vomiting, persistent gastric secretion and cystic fibrosis.

***** End Of Report *****


DR. SHARWARI NARAWADE
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Reg. No: 2012030602

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DEPARTMENT OF PATHOLOGY
TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260404BI0301
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY (ROUTINE)

Serum Amylase		Sample Id		: 0310BI260404
Collection DateTime	: 04/04/2026 09.15.34	Receiving	: 04/04/2026 10.35.58	Reporting DateTime : 06/04/2026 15.15.20
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status : Result Certified
Test	Result	Method	Unit	Reference Interval
Serum Amylase	18	CNPG3	IU/L	28.0 - 100.0

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Increased in acute pancreatitis, chronic pancreatitis, cancer of pancreas,pancreatic pseudocyst ascites, perforated ulcer induced pancreatitis intestinal obstruction,acute cholecystitis, peritonitis. Decreased in chronic pancreatitis, liver failure, cystic fibrosis

***** End Of Report *****


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H-2019-0663
Since Sep 24, 2019

DEPARTMENT OF PATHOLOGY
TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260404HM0234
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

HAEMATOLOGY

Haemogram[Panel]	Sample Id	: 0199HM260404
Collection DateTime : 04/04/2026 09.15.35	Receiving : 04/04/2026 10.15.41	Reporting DateTime : 04/04/2026 12.38.38
Primary Sample Type : Whole Blood	Sample Suitability : Suitable	Result Status : Result Certified

Test	Result	Method	Unit	Reference Interval
Haemoglobin	8.7	SLS	gm %	13.0 - 18.0
RBC Count	3.49	Impedance	Mill/Cmm	4.0 - 5.5
PCV	29.2	Cumulative pulse height detection	%	40.0 - 50.0
MCV	83.7	Calculated	fL	75.0 - 95.0
MCH	24.9	Calculated	pg	25.0 - 32.0
MCHC	29.8	Calculated	g/dl	30.0 - 35.0
RDW	22.8	Calculated	%	11.60 - 14.80
Total Leucocyte Count ..	12290	Fluorescence FCM	/Cmm	4000.0 - 10000.0 ***** FCM= Flow Cytometry
.Differential Count.				
Neutrophils	85.6	Fluorescence FCM	%	40.0 - 75.0
Lymphocytes-	8.0	Fluorescence FCM	%	20.0 - 45.0
Eosinophils	0.1	Fluorescence FCM	%	1.0 - 6.0
Monocytes	6.2	Fluorescence FCM	%	1.0 - 10.0
Basophils	0.1	Fluorescence FCM	%	0.1 - 1.0
Absolute Neutrophil Count (ANC)	10530	Fluorescence FCM	/Cmm	2000.0 - 7500.0
Absolute Lymphocytes Count	980	Fluorescence FCM	/Cmm	1500.0 - 4000.0
Absolute Eosinophil Count (AEC)	10	Fluorescence FCM	/Cmm	40.0 - 400.0
Absolute Monocytes Count	760	Fluorescence FCM	/Cmm	200.0 - 800.0
Absolute Basophil Count	10	Fluorescence FCM	/Cmm	0.1 - 100.0
Platelet Count	292000	Impedance	/Cmm	150000 - 450000
MPV	9.1	Calculated	fL	8 - 11.5

.Peripheral Smear				
RBCs..-	Normocytic normochromic, moderate anisocytosis			
WBCs (Hgm)	Neutrophilic leucocytosis			
Platelet Morphology	Adequate			

NOTE : All abnormal haemogram are reviewed and confirm microscopically

***** End Of Report *****

A.V. Bharati
DR. ANKITA BHARATI
MBBS, MD Pathology
Senior Registrar
Reg. No.:2020/05/3263



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DEPARTMENT OF PATHOLOGY

TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Ward/Room	:SS-7 th floor A2 wing
Category	:TPA-PPN	Lab Reference No	:20260404HM0234
		Referred By	:DR. KURLEKAR UTKRANT

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Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260407BI0081
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY (SPECIAL)

Lipase				Sample Id	: 0094BI260407
Collection DateTime	: 07/04/2026 05.58.09	Receiving	: 07/04/2026 07.33.30	Reporting DateTime	: 07/04/2026 17.03.17
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
Lipase	83	Kinetic colorimetric	U/L	<= 60.0 U/L	

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Increased in, acute pancreatitis, sudden inflammation of the pancreas, chronic pancreatitis, long-term inflammation of the pancreas, pancreatic pseudocyst, fluid-filled sac around the pancreas, pancreatic cancer, cholecystitis, inflammation of the gallbladder, ectopic pregnancy, mumps, salivary gland blockage, intestinal blockage. Decreased in, severe injury to the pancreas, high triglycerides, diabetes

***** End Of Report *****


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MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260407BI0081
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY(ROUTINE)

Liver Function Tests[Panel]	Sample Id	: 0094BI260407
Collection DateTime : 07/04/2026 05.58.09	Receiving	: 07/04/2026 07.33.30
Primary Sample Type : Serum	Sample Suitability	: Suitable
	Reporting DateTime	: 07/04/2026 16.41.20
	Result Status	: Result Certified

Test	Result	Method	Unit	Reference Interval
*Liver Function Test:	.			
Total Bilirubin	0.38	DMSO	mg %	0.2 - 1.2 Cord blood : < 2.0
Direct Bilirubin	0.20	DMSO	mg %	Upto 0.5
Indirect Bilirubin	0.18	Calculated	mg %	Upto 0.8
SGPT	8	(Kinetic)	IU/L	10.0 - 40.0
SGOT	9	(Kinetic)	IU/L	11.0 - 34.0
Serum Alkaline Phosphatase	58	PNP-AMP	IU/L	50.0 - 116.0
Total Proteins	4.24	Biuret	gm %	6.0 - 8.0
Serum Albumin	2.91	BCG	gm %	3.5 - 5.2
Serum Globulin	1.33	Calculated	gm %	2.0 - 3.5
Albumin: Globulin Ratio	2.19	Calculated	-	1.0 - 2.0

HIL INDEX-WNL

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Interpretation: Increased AST and ALT levels indicate liver damage, disease or muscle damage. Increased ALP indicate liver damage or blocked bile duct, or certain bone diseases. Elevated levels of bilirubin (jaundice) indicates liver damage or disease or certain types of anemia. Lower-than-normal levels of albumin and total protein may indicate liver damage or disease.High bilirubin and normal direct bilirubin indicate hemolysis. Similar situation is in neonates. Increased bilirubin and increased ALP with near normal AST and ALT suggests obstructive jaundice.

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MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260407BI0081
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY (ROUTINE)

RFT[Panel]		Sample Id		: 0094BI260407	
Collection DateTime	: 07/04/2026 05.58.09	Receiving	: 07/04/2026 07.33.30	Reporting DateTime	: 07/04/2026 16.26.10
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
Blood Urea Level	21	(UV Kinetic)	mg %	19.44 - 44.08	
Creatinine::	0.60	(Alkaline picrate)	mg %	0.6 - 1.3	
eGFR	153.49	Calculated	ml/min	Normal/Minimal kidney damage:>=90 ml/min/1.73m2 Impairment : ml/min/1.73m2 Mildly decreased : 60-89 Mild to moderately decreased: 45-59 Moderately to severely decreased : 30-44 Severely decreased:15-29	
Serum Calcium	7.65	(Arsenazo III)	mg %	8.4 - 10.2	
Serum Phosphorus	2.20	(UV phosphomolybdate)	mg %	2.3 - 4.7	
Serum Uric Acid	4.19	(Uricase)	mg %	3.7 - 7.7	
Serum Sodium	136	ISE(Indirect)	mEq/L	136.0 - 145.0	
Serum Potassium	3.7	ISE(Indirect)	mEq/L	3.5 - 5.1	
Serum Chlorides	105	ISE(Indirect)	mEq/L	98.0 - 107.0	

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Serum urea is increased in gastrointestinal bleeding, dehydration, kidney disease, obstruction in the urinary tract, and shock. Decreased with liver failure, malnutrition.Serum creatinine increased in acute and chronic renal failures, shock, severe dehydration and muscular waste due to severe injury. Decreased in protein-calorie malnutrition. High total calcium may be due to Hyperthyroidism. Low total calcium is due to hypoparathyroidism, extreme deficiency in dietary calcium. Increase in phosphate is due to high intake of phosphate or vitamin D, hypocalcemia, hypoparathyroidism. Decreased in chronic use of antacids, lack of vitamin D. Uric acid is increased in gout, renal failure, leukemia, Lesch-Nyhan syndrome. Decreased in Wilson’s disease, Fanconi syndrome. Sodium is increased in water loss, dehydration, vomiting, diarrhea, burns. Decreased in use of diuretics, renal tubular acidosis. Potassium is elevated with increased supply of potassium as in potassium infusion, redistribution of potassium and decreased with chronic starvation, prolonged vomiting, diarrhea. Chloride is increased in renal tubular acidosis, acute renal failure, diabetes insipidus and decreased in excessive sweating, prolonged vomiting, persistent gastric secretion and cystic fibrosis.

***** End Of Report *****

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Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260407BI0081
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

BIOCHEMISTRY (ROUTINE)

Serum Amylase		Sample Id		: 0094BI260407
Collection DateTime	: 07/04/2026 05.58.09	Receiving	: 07/04/2026 07.33.30	Reporting DateTime : 07/04/2026 17.03.17
Primary Sample Type	: Serum	Sample Suitability	: Suitable	Result Status : Result Certified
Test	Result	Method	Unit	Reference Interval
Serum Amylase	66	CNPG3	IU/L	28.0 - 100.0

HIL INDEX-WNL

Abbreviation: H-Hemolysis,I-Icterus,L-Lipemia,WNL-Within normal limits.

Interpretation: Increased in acute pancreatitis, chronic pancreatitis, cancer of pancreas,pancreatic pseudocyst ascites, perforated ulcer induced pancreatitis intestinal obstruction,acute cholecystitis, peritonitis. Decreased in chronic pancreatitis, liver failure, cystic fibrosis

***** End Of Report *****


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DEPARTMENT OF PATHOLOGY

TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Lab Reference No	:20260407HM0063
Category	:TPA-PPN	Ward/Room	:SS-7 th floor A2 wing
		Referred By	:DR. KURLEKAR UTKRANT

HAEMATOLOGY

Haemogram[Panel]				Sample Id	: 0086HM260407
Collection DateTime	: 07/04/2026 05.58.09	Receiving	: 07/04/2026 07.16.20	Reporting DateTime	: 07/04/2026 12.41.07
Primary Sample Type	: Whole Blood	Sample Suitability	: Suitable	Result Status	: Result Certified
Test	Result	Method	Unit	Reference Interval	
Haemoglobin	8.6	SLS	gm %	13.0 - 18.0	
RBC Count	3.42	Impedance	Mill/Cmm	4.0 - 5.5	
PCV	28.7	Cumulative pulse height detection	%	40.0 - 50.0	
MCV	83.9	Calculated	fL	75.0 - 95.0	
MCH	25.1	Calculated	pg	25.0 - 32.0	
MCHC	30.0	Calculated	g/dl	30.0 - 35.0	
RDW	22.2	Calculated	%	11.60 - 14.80	
Total Leucocyte Count ..	7770	Fluoresence FCM	/Cmm	4000.0 - 10000.0***** FCM= Flow Cytometry	
<u>.Differential Count.</u>	.				
Neutrophils	65.4	Fluoresence FCM	%	40.0 - 75.0	
Lymphocytes-	16.0	Fluoresence FCM	%	20.0 - 45.0	
Eosinophils	9.5	Fluoresence FCM	%	1.0 - 6.0	
Monocytes	8.6	Fluoresence FCM	%	1.0 - 10.0	
Basophils	0.5	Fluoresence FCM	%	0.1 - 1.0	
Absolute Neutrophil Count (ANC)	5080	Fluoresence FCM	/Cmm	2000.0 - 7500.0	
Absolute Lymphocytes Count	1240	Fluoresence FCM	/Cmm	1500.0 - 4000.0	
Absolute Eosinophil Count (AEC)	740	Fluoresence FCM	/Cmm	40.0 - 400.0	
Absolute Monocytes Count	670	Fluoresence FCM	/Cmm	200.0 - 800.0	
Absolute Basophil Count	40	Fluoresence FCM	/Cmm	0.1 - 100.0	
Platelet Count	301000	Impedance	/Cmm	150000 - 450000	
MPV	9.1	Calculated	fL	8 - 11.5	
<u>.Peripheral Smear</u>	.				
RBCs..-	Normocytic normochromic, moderate anisocytosis				
WBCs (Hgm)	Mild eosinophilia				
Platelet Morphology	Adequate				

NOTE : All abnormal haemogram are reviewed and confirm microscopically

***** End Of Report *****



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DEPARTMENT OF PATHOLOGY

TEST REPORT

Name	:Mr. HOLE NITIN SHIVAJI	Printing DateTime	:14/05/2026 16.09.30
MRD No	:332192	Visit Code	:IP0001
DoB/Sex	:20/06/1978 [47Y 9M] : M	Ward/Room	:SS-7 th floor A2 wing
Category	:TPA-PPN	Lab Reference No	:20260407HM0063
		Referred By	:DR. KURLEKAR UTKRANT

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Patient Name: Mr. HOLE NITIN SHIVAJI

MRD#: 332192

Date of Birth: 20/06/1978

Sex: Male

Visit Code: IP0002

Created Date: 08/05/2026

Speciality: ONCOLOGY

Ward/Bed No: SS7C2 - 3746

Consultant: Dr. DESHMUKH CHETAN

Date of Admission: 08/05/2026

Date of Discharge: 10-05-2026

Blood Group: AB POS

Discharging Status: FOLLOW UP DISCHARGE SUMMARY

DIAGNOSIS :

Carcinoma of colon

MEDICATION ON ADMISSION:

Inj Leucovorin 700mg

Inj 5 FU 3750mg

HISTORY OF PRESENT ILLNESS:

Ref by Dr. Kurlekar

47 yo male

HTN

Mother-Breast cancer survivor

Evaluated for loose stools, anemia in December 2025

4/3/26 2D echo Bicuspid AV?, Conc LVH,EF 65% (not seen in 2024 echo)

Colonoscopy done on 20/3/2026: Polyposis coli- endoscopically adenomatous
Growth at the ascending colon (by extinsic compression), likely consideration of pan colectomy with a distal 5 cm margin.

Biopsy on 24/3/2026: Well-differentiated invasive adenocarcinoma of right colon with multiple tubulovillous adenomatous polyps with moderate to focally severe dysplasia.

PET CT on 21/3/2026: Metabolically active circumferential asymmetric wall thickening at ascending colon involving hepatic flexure and extending to proximal transverse colon as described with colo-colic intussusception. Multiple metabolically active broad based polypoidal lesion in rest of the colon

Operated by Dr. Kurlekar on 2/4/26

Diagnostic laparoscopy +laparotomy + Total colectomy +ileo- rectal side to end circular stapler anastomosis + mini segment 6 hepatectomy + intral operative colonoscopy

HPE S/o--

A) Total colectomy:

- Moderately Differentiated Adenocarcinoma of Right colon, infiltrating the subserosal fat.
- Background colon show multiple colonic polyps with features of Tubular & Tubulovillous adenomas with low grade dysplasia.
- All 18 lymph nodes are reactive.
- Both resection margins are unremarkable.

B) Both Lymph nodes at root of ileocolic vessel are reactive. No dysplasia or malignancy.

MRD No:332192

Name:Mr. HOLE NITIN SHIVAJI

C) Distal donut also shows tubular adenoma with low grade dysplasia.
No evidence of high grade dysplasia or malignancy.
TNM stage: pT3N0

7/4/26 CBC 8.6/7770/301000
Creat 0.6
Alb 2.8
Rest LFT,SE WNL

Stitches removed on 17/4/26

MMR proficient by IHC(MSI-Stable phenotype)

Plan--Considering family history and MSI-stable phenotype, to offer adjuvant chemotherapy
Check ser. DPD level

Now admitted for Cycle 1st 5 FU + Leucovorin on 08/05/26

INVESTIGATIONS:

COURSE IN THE HOSPITAL AND DISCUSSION:

Tolerated chemo well
PICC insertion in next cycle
Discharge in stable condition

PLAN ON DISCHARGE:

Follow-up on 21/05/26 in OPD for PICC line insertion

Do CBC, RFT, LFT, Sr. Electrolytes on 20/05/26 and inform on whatsapp
Admission for Next chemo on 22/05/26 under Dr.Chetan Deshmukh

DISCHARGE PRESCRIPTION:

Inj Adfill 6mg 1--0--0 s/c x 1 day on 12/05/26

Tb Pan 40mg 1--0--0 x 5 days (before food)
Tb Domstal 10mg 1--1--1 x 5 days (before food)
Syp Mucaïne gel 5ml 1--1--1 x 5 days (before food)
Tb Folvite 1--0--1 x 15 days
T Ultracet 1--0--1 x 4 days
Syp Duphalac 30ml x sos if required for constipation

ADVICE ON DISCHARGE:

Inform immediately if any emergency like fever / nausea / vomiting etc and kindly approach Deenanath Mangeshkar Hospital CASUALTY or any nearby hospital.

CONTACT DETAILS:

For any assistance or to share patient reports WhatsApp on 8999536998 in between 9.00 am - 4.30 pm on working days only (Monday to Saturday) with following details

MRD Number Patient Name Consultant Name

Dept. E-mail: dmhmedoncology@gmail.com, oncology@dmhospital.org

For Appointments: - 020 40 15 11 00 , Hospital Contact: 020 40151000 / 020 49153000

OPD Days and Timings (By Appointment only): Dr. Chetan Deshmukh (Tuesday and Thursday 10am to 5pm)

DIET RECOMMENDATIONS:

. Normal diet, . Avoid hot and spicy food

SPECIAL NEEDS:

Oral mucositis prevention care:

. Use mouth wash after every meal

Neutropenia and infection prevention care:

MRD No:332192

Name:Mr. HOLE NITIN SHIVAJI

. Avoid contact with persons having fever or cough or cold or other signs of infection, . Maintain personal hygiene, . Consume plenty of boiled/filtered water. or green leafy vegetables. Use fruits cleaned with water and after peeling.

Fever while on chemotherapy - instructions:

Report immediately to oncology team or family physician or MD physician or nearby hospital.

SIGNED BY: Dr. DESHMUKH CHETAN

PREPARED BY: DR.DESHPANDE RUTURAJ SADANAND





DEPARTMENT OF PATHOLOGY
TEST REPORT

Printed Date:14/05/2026 16:09:48

Histopathology report

Patient Name: Mr. HOLE NITIN SHIVAJI

MRD#: 332192

Date of Birth: 20/06/1978

Sex: Male

Visit Code: OP0010

Sample Id: 0027HS260421

Referred By: Dr. DESHMUKH CHETAN

Sample Suitability: Suitable

Result status: RC

Collection Date/Time: 21-04-2026 14:23

Receiving Date/Time: 21-04-2026 14:23

Result Date/Time: 25-04-2026 10:20

Sample ID: 0027HS260421

IMMUNOHISTOCHEMISTRY REPORT

Specimen No: D26/4128/A4

Specimen:

Total colectomy for MSI by IHC.

Clinical:

A) Total colectomy:

- Moderately Differentiated Adenocarcinoma of Right colon, infiltrating the subserosal fat.
- Background colon show multiple colonic polyps with features of Tubular & Tubulovillous adenomas with low grade dysplasia.
- All 18 lymph nodes are reactive.- Both resection margins are unremarkable.

B) Both Lymph nodes at root of ileocolic vessel are reactive. No dysplasia or malignancy.

C) Distal donut also shows tubular adenoma with low grade dysplasia.
No evidence of high grade dysplasia or malignancy.

* TNMpath stage: pT3N0

Test:

MSI by IHC.

IHC Result:

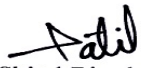
The neoplastic cells show Strong expression of MLH1, PMS2, MSH2 and MSH6 (positive internal control).

Impression:

The immunoprofile is suggestive of Proficiency in mismatch repair protein/normal MMR function. Consistent with Microsatellite stable (MSS) phenotype.

Remark/Note:

* The internal & external controls have been satisfactory.



Dr. Shital Biradar
DNB (Pathology)
Fellowship in GI Pathology
Consultant GI Pathologist
Reg No:2006/02/1150

Signed By: svb03



MRD No:332192

Name:Mr. HOLE NITIN SHIVAJI

[Scan QR Code For - Patient Portal]



Note : Interpretation of Lab Results may vary in the light of clinical data.
Kindly correlate clinically & communicate your queries if any.





SURGICAL DISCHARGE SUMMARY

Patient Name: Mr. HOLE NITIN SHIVAJI

MRD#: 332192

Date of Birth: 20/06/1978

Sex: Male

Visit Code: IP0001

Created Date: 10/04/2026

Speciality: SURGERY

Ward/Bed No: SS7A2 - 3710

Consultant: Dr. KURLEKAR UTKRANT

Date of Admission: 31/03/2026

Date of Procedure: 02/04/2026

Date of Discharge: 10/04/2026

Blood Group: AB POS

Weight in Kg: 93.5

Height in Cm: 178

Age: 47Y 9M 20D

Discharging Status: FOLLOW UP DISCHARGE SUMMARY

DIAGNOSIS :

Adenocarcinoma colon

PROCEDURE DONE:

SURGEON:

Dr. KURLEKAR UTKRANT

Dr. WAGHOLIKAR GAJANAN

ANAESTHETIST:

Dr. KULKARNI KETAN SAKHARAM

ANAESTHESIA: General+Epidural

SURGERY/PROCEDURE PERFORMED: Diagnostic laparoscopy +laparotomy + Total colectomy +ileo-rectal side to end circular stapler anastomosis + mini segment 6 hepatectomy + intral operative colonoscopy

DRUG ALLERGIES:

No known drug allergies

MEDICATION ON ADMISSION:

Peglec bowel preperation

IV fluids

Inj. Albumin

HISTORY OF PRESENT ILLNESS:

47 year old male patient came with complaints of loose stools since associated with loss of weight. Colonoscopy done on 20/3/2026: Polyposis coli- endoscopically adenomatous Growth at the ascending colon (by extinsic compression), likely consideration of pan colectomy with a distal 5 cm margin. Biopsy on 24/3/2026: Well-differentiated invasive adenocarcinoma of right colon with multiple tubulovillous adenomatous polyps with moderate to focally severe dysplasia.

PET CT on 21/3/2026: Metabolically active circumferential asymmetric wall thickening at ascending colon involving hepatic flexure and extending to proximal transverse colon as described with colo-colic intussusception. Multiple metabolically active broad based polypoidal lesion in rest of the colon as described.

Admitted for further management.

PERSONAL HISTORY:

Known case of Hypertension, on medications

CLINICAL EXAMINATION:

Patient alert and responsive

Vital:

Temperature : 96.9° F

pulse: 72 beats per minute
Respiratory rate : 20 cycles per minute
BP: 111/83 mmHg
SPO2: 97% on room air
RS: AEBE
CVS : S1, S2 heard, no murmurs
CNS : NAD
PA: Soft, non tender

COURSE IN THE HOSPITAL AND DISCUSSION:

Patient was admitted with above mentioned complaints. He underwent Colonoscopy and findings were noted. He underwent surgery and tolerated the procedure well. He was treated with analgesics, antibiotics and other supportive measures. He ambulated well in ward. He was kept nil by mouth post surgery and gradually started on sips of water followed by clear liquids, liquid diet, soft diet and full diet, which was tolerated well. Drain outputs were monitored daily and drains removed accordingly. Rest of the hospital course went uneventful. Patient is being discharged in stable condition with ADK drain insitu.

OPERATIVE FINDINGS:

A large circumferential growth was identified at the hepatic flexure, extending from the distal ascending colon to the proximal transverse colon, associated with multiple enlarged perilesional lymph nodes. The lesion was densely adherent to segment VI of the liver. The remaining liver surface appeared grossly normal. Multiple polyps were noted throughout the colon. Intraoperative colonoscopy was performed to determine the distal extent of disease, which revealed multiple sessile and pedunculated polyps in the distal sigmoid colon. A rectal polyp was also identified; however, a decision was made to polypectomy later and keep the patient under close postoperative surveillance. There was no evidence of peritoneal metastasis. The small bowel appeared grossly normal

PLAN ON DISCHARGE:

Follow up with Dr Kurlekar in Surgery OPD, GS building ground floor D wing on Tuesday 14/4/26 between 10 am and 1 pm.

Follow up with Dr Iyer in Medicine OPD after 1 week.

DISCHARGE MEDICATION:

Tab. Levoflox 500 mg 0-1-0 after food x 7 days
Tab. Pan 40 mg 1-0-1 before food x 7 days
Tab. Combiflam 1-1-1 after food x 3 days, then if required for pain
Cap. Becozinc 0-1-0 after food x 15 days
Syp. Cremaffin 30 ml at bedtime x 1 month
Nakpro protein powder 1 scoop in 1 glass milk/water 1-0-1 x 1 month

From medicine team:

Tab Tenormin 50 mg 1-0-0

CONTACT DETAILS :

For any assistance or to share patient reports, WhatsApp on 9226223649 between 9.30 am to 5.00 pm on working days only (Monday to Saturday)

Dept. E-mail: surgery@dmhospital.org, dmhsurgery@gmail.com

For Appointments: 020-40151100

Surgery OPD: 020-40151084 (Main building, Ground Floor)

OncoSurgery OPD - 020-40151000 / 020-49153000 (Annexe Building, Ground Floor)

ADVICE ON DISCHARGE:

In case of emergency (Bleeding, Fever, Pain, Wound discharge, Vomiting, Constipation, Difficulty in Breathing), please come to ER 1 (Emergency Room). It is open 24 hours a day.

Special needs:

Diet plan:
Regular diet

Dressing:

Keep dressing dry.

Special Needs if any:

Use abdominal binder.

Avoid lifting heavy weights.

Avoid exercises which put strain on abdomen.

Avoid straining for passing motions.

Permitted physical activity: Walking

Drain:

ADK drain insitu. Drain emptying, measurement and care to be done as taught to patient and relatives.

SIGNED BY: Dr. KURLEKAR UTKRANT

PREPARED BY: DR.JOSHI PRANAV SUNIL





DEPARTMENT OF PATHOLOGY
TEST REPORT

Printed Date:14/05/2026 16:09:49

Patient Name: Mr. HOLE NITIN SHIVAJI

MRD#: 332192

Date of Birth: 20/06/1978

Sex: Male

Visit Code: IP0001

Ward/Bed: SS7A2-3710

Referred by: Dr. KURLEKAR UTKRANT

Sample Suitability: Suitable

Report Status: RC

Receiving Date/Time: 02-04-2026 17:40

Collection Date/Time: 02-04-2026 16:19

Result Date/Time: 08-04-2026 12:49

Sample ID: 0033HS260402

SURGICAL PATHOLOGY REPORT

Specimen No:

D26/4128

Clinical History:

C/o Ca colon

Specimen:

- A) Total colectomy in formalin is a specimen for HPE.
- B) Lymph node tissue at root of ileo-colic vessel for HPE.
- C) Distal donut for HPE.

Gross:

A) Received in formalin is a specimen labeled as "total colectomy" consists of segment of small intestine measuring 14 cm in length, caecum & part of ascending colon measuring 76 cm in length.

Appendix measures 5.5 cm in length.

External surface is unremarkable.

Luminal aspect shows an ulceroproiferative circumferential grayish white polypoidal tumour measuring 8 x 11 x 4 cm in the caecum and ascending colon.

Tumour is invading the wall up to subserosal fat.

Tumour site perforation is absent.

Rest of colonic mucosa shows multiple polyps approximately more than 50 in numbers.

Largest polyp measuring 2.5 x 2 x 1.2 cm.

Ileum & appendix are unremarkable.

Resection margins:

Tumour is 22 cm from proximal end (small intestine) and 68 cm from distal resection margin (Colonic).

Regional lymph nodes:

Total 18 regional lymph nodes are dissected out. Largest lymph node measures 2.5 x 1.5 x 1.0 cm.

(A1 to A3-Tumor composite, A4, A5-Tumor composite, A6, A7-Tumor composite, A8 to A10 -Tumor composite, A11, A12 -Tumor composite, A13 - Appendix, A14 to A19 -polyp, A20 - polyp, A21 - PRM, A22 - DRM, A23- small intestine, A24- caecum, A25- colon (representative section), A26-A27- largest LN, A28-A35: lymph nodes.)

B) Received in formalin is a specimen labeled as "Lymph node tissue at root of ileo-colic vessel" consists of the same measuring 2.2 x 1.5 x 0.8 cm. B1- All processed.

MRD No:332192

Name:Mr. HOLE NITIN SHIVAJI

C) Received in formalin is a specimen labeled as "Distal donut" consists of two tissue pieces together measuring 1.5 x 1.2 x 1.0 cm. C1: All processed.

Microscopy:

A) Total colectomy shows features of Moderately Differentiated Adenocarcinoma infiltrating the subserosal fat.

Background show multiple colonic polyps with features of Tubular & Tubulovillous adenomas with low grade dysplasia.

Type of tumour: Adenocarcinoma, moderately differentiated.

Histologic Grade: Grade II

Tumor Extension: Tumor is seen infiltrating the subserosal fat.

Tumour Budding: Not Seen

Lymphovascular emboli: Not seen

Perineural invasion: Not seen

Extramural tumour deposits: Not seen

Background abnormalities: colon shows multiple Tubular & Tubulovillous adenomas with low grade dysplasia.

Ileum & Appendix are unremarkable.

Lymph nodes :

Total number of lymph nodes identified: 18

Total number of lymph nodes involved by tumour: 00

Resection margins:

Proximal resection margin, distal resection margin both are unremarkable.

B) Both Lymph nodes at root of ileocolic vessel are reactive. No dysplasia or malignancy.

C) Distal donut also shows small tubular adenoma with low grade dysplasia.

No evidence of high grade dysplasia or malignancy.

Final Diagnosis:

A) Total colectomy:

- Moderately Differentiated Adenocarcinoma of Right colon, infiltrating the subserosal fat.
- Background colon show multiple colonic polyps with features of Tubular & Tubulovillous adenomas with low grade dysplasia.
- All 18 lymph nodes are reactive.
- Both resection margins are unremarkable.

B) Both Lymph nodes at root of ileocolic vessel are reactive. No dysplasia or malignancy.

C) Distal donut also shows tubular adenoma with low grade dysplasia.

No evidence of high grade dysplasia or malignancy.

* TNM path stage: pT3N0.

*******End of Report*******



Dr. Shital Biradar
DNB (Pathology)
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Reg No:2006/02/1150

Signed By: svb03

MRD No:332192

Name:Mr. HOLE NITIN SHIVAJI

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Note : Interpretation of Lab Results may vary in the light of clinical data.
Kindly correlate clinically & communicate your queries if any.





DEPARTMENT OF PATHOLOGY
TEST REPORT

Printed Date:14/05/2026 16:09:49

Patient Name: Mr. HOLE NITIN SHIVAJI

MRD#: 332192

Date of Birth: 20/06/1978

Sex: Male

Visit Code: IP0001

Ward/Bed: SS7A2-3710

Referred by: Dr. KURLEKAR UTKRANT

Sample Suitability: Suitable

Report Status: RC

Receiving Date/Time: 01-04-2026 16:04

Collection Date/Time: 01-04-2026 12:20

Result Date/Time: 02-04-2026 15:15

Sample ID: 0016HS260401

SURGICAL PATHOLOGY REPORT

Specimen No:

D26/4068

Clinical History:

C/o FAP

Specimen:

Gastric polyp biopsy for HPE.

Gross:

Received in formalin is a specimen consisting of multiple whitish soft tissue bits together measuring 0.7 x 0.5 x 0.3 cm. (A1-All processed.)

Microscopy:

Gastric polyp biopsy shows features of Fundic gland polyps.
No dysplasia or malignancy.

Final Diagnosis:

Gastric polyp biopsy:
Fundic gland polyps(benign).
No dysplasia or malignancy.

*******End of Report*******

Dr. Shital Biradar
DNB (Pathology)
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Reg No:2006/02/1150

Signed By: svb03

MRD No:332192

Name:Mr. HOLE NITIN SHIVAJI

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